

India's largest Health Test @Home Service

India's Most Awarded Healthcare Brand



Booking ID : 5316220876

Rajesh Kulkarni

Male, 26 Years

A Comprehensive Health Analysis Report

AI Based Personalized Report for You



INDIA'S FIRST & ONLY CREDIBILITY CHECK FOR YOUR LAB REPORT

Check the authenticity of your lab report with machine data

Scan the QR using any QR code scanner or alternatively follow below steps :



Go to bit.ly/verifyqr on your mobile



Scan the QR Code

Rajesh Kulkarni | Booking ID : 5316220876

Healthians Smart Report

A Self explanatory Health Diagnostics Report

Healthians Smart report is **India's most innovative** and easy to understand report that describes all information in an intuitive way required for **better health & lifestyle** of customers

Below are the sections which depict what you can expect from this report , how you can read this report and use it for your well-being.

1. Health Analysis

This section summarizes your test results, your critical health parameters and on basis of them where you should draw your attention to. This has been determined by lab results & health karma questions which you answered regarding your lifestyle.



2. Historical Charts

These charts are a way to measure and keep a track of how your health has progressed over time. We depict important parameters here and depending on your test history, the charts describe rise and fall of your health metrics.



3. Lab Test Results

Comprehensive test results generated through use of latest technology and quality checks by health experts. This section provides an exhaustive view of which tests you have taken, ideal result and your actual result with highlighted focus points.



4. Health Advisory

An Advisory section suggesting what modifications to bring in your nutrition & lifestyle, recommendations on your BMI along with regular tests and further consultations to pursue for a healthier future.



5. General Recommendations

Brief view of general preventive test recommendations categorized by age groups. Refer this section to know at what age, which tests are necessary and at what frequency they should be booked.



Disclaimer:

- This report is not intended to replace but to lead by providing comprehensive information. It is recommended that you consult your doctor/physician for interpretation of results.
- All reports might not be applicable for individuals less than 18, pregnant women or individuals suffering from diseases for which health test has not been performed or symptoms not diagnosed.
- This report is based on preventive health test screening and is meant for a healthy lifestyle. It does not provide any recommendation for life threatening situations.
- It is strongly recommended to take required precautions for allergic reactions or sensitivities.

HEALTH ANALYSIS

Personalized Summary & Vital Parameters

Rajesh Kulkarni

Booking ID : 5316220876

Rajesh Kulkarni,

Congratulations, We have successfully completed your health diagnosis. This is a big step towards staying on top of your health and identify potential to improve!

10 Vital Health Parameters of a Human Body Ecosystem

Below are the health parameters which require routine checkups for primary healthcare. The view also includes *personalised information* depending on the tests you have taken.

Comorbidities: Yes

* Adults of any age with Comorbidities are at increased risk of severe illness from the virus that causes COVID-19.



Your Health Score

79
Out of 100

*Calculated from test reports



Thyroid Function

Thyroid Stimulating Hormone (TSH)-Ultrasensit : 3.2060 µIU/ml

• Everything looks good



Cholesterol Total

157 mg/dl

• Everything looks good



Kidney Function

Serum Creatinine : 0.73 mg/dl

• Everything looks good



Vitamin D

11.37 ng/ml

• Concern



HbA1c

9.90 %

• Concern



Vitamin B12

503 pg/ml

• Everything looks good



Liver Function

Alanine Aminotransferase (ALT/SGPT) : 11.00 U/l

• Everything looks good



Calcium Total

9.8 mg/dl

• Everything looks good



Iron studies

Serum Iron : 16.4 ug/dl

• Concern



Complete Hemogram

Haemoglobin (HB) : 11.2 g/dl

• Concern



HEALTH ANALYSIS

Critical Parameters

Rajesh

Kulkarni Booking ID :
5316220876

We have observed that the below given critical parameters have shown out of range results, which can have negative impact on your health.

Chlorides, Serum

Serum Chloride test is essential to monitor the level of chlorides in the body. Chloride is carefully controlled by the kidneys and is required for metabolism. It also helps maintain the body's blood volume, blood pressure and acid-base balance.

Impact on overall health?

This test can assess your risk of imbalance of chlorides or electrolytes, disturbed metabolism, dehydration, blood pressure, kidney functioning and acid-base balance problems.

How to improve health conditions?

A healthy balanced diet with plenty of water, fresh fruits and vegetables can help maintain electrolyte balance. Oral rehydration can help in low chloride levels.

Your Result Value

↑ **110** mmol/L

Concern

Normal Value

● **99-109** mmol/L

HEALTH ANALYSIS

HISTORICAL CHARTS

Rajesh Kulkarni

Booking ID : 5316220876

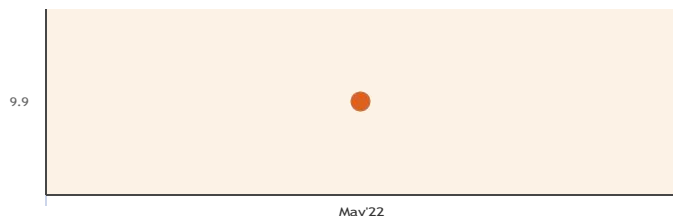
Glycated Hemoglobin (HbA1c)

Concern

Your Latest result

9.90 %

18th May 2022



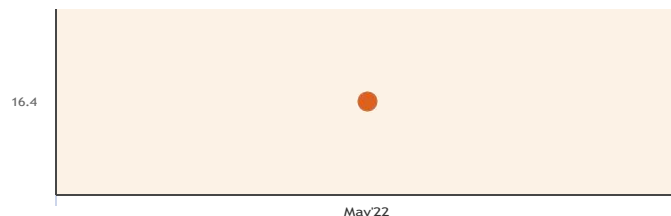
Iron, Serum

Concern

Your Latest result

16.4 ug/dl

18th May 2022



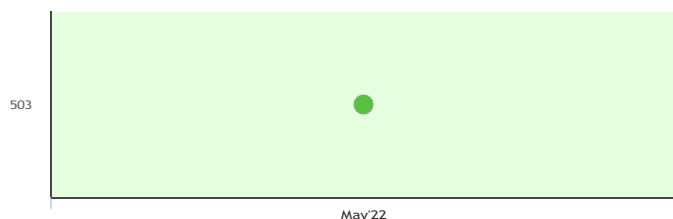
Vitamin B12 Cyanocobalamin

Everything looks good

Your Latest result

503 pg/ml

18th May 2022



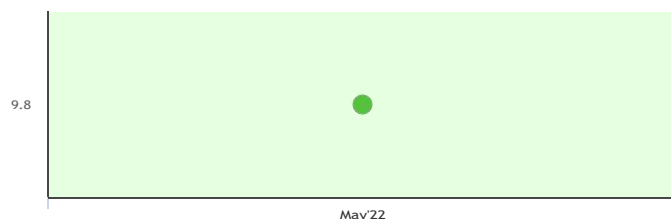
Calcium Total, Serum

Everything looks good

Your Latest result

9.8 mg/dl

18th May 2022



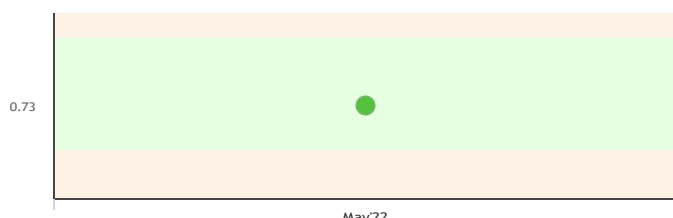
Creatinine, Serum

Everything looks good

Your Latest result

0.73 mg/dl

18th May 2022



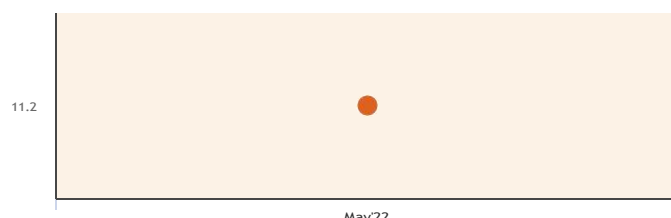
Hemoglobin Hb

Concern

Your Latest result

11.2 g/dl

18th May 2022



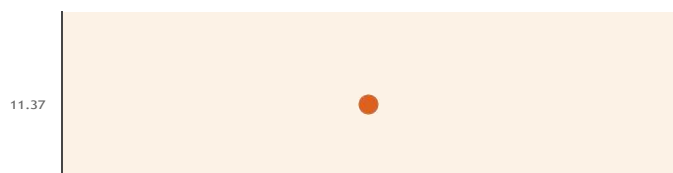
Vitamin D Total-25 Hydroxy

Concern

Your Latest result

11.37 ng/ml

18th May 2022



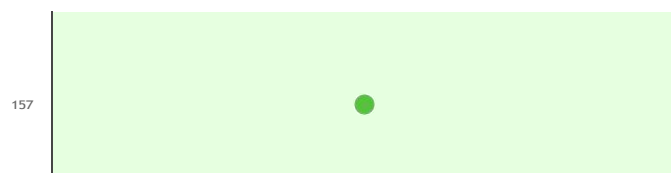
Cholesterol-Total, Serum

Everything looks good

Your Latest result

157 mg/dl

18th May 2022



Patient Name	: Rajesh Kulkarni	Barcode	: H6026393	
5316220876		Sample Collected On	: 18/Sept/2025 07:00AM	
Age/Gender	: 26/Male	Sample Received On	: 18/Sept/2025 11:22AM	
Order Id	: 5316220876	Report Generated On	: 18/Sept/2025 02:22PM	
Referred By	: Self	Sample Temperature	: Maintained ✓	
Customer Since	: 18/Sept/2025	Report Status	: Final Report	
Sample Type	: Whole Blood EDTA			

DEPARTMENT OF BIOCHEMISTRY HBA1C

Test Name	Value	Unit	Bio. Ref Interval
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HbA1c - Glycated Hemoglobin

HbA1c (Glycosylated Hemoglobin)	11.90	%	4.2 - 5.7
Method: HPLC			
Average Estimated Glucose - plasma	243.43	mg/dl	
Method: Calculated			

AMERICAN DIABETES ASSOCIATION (ADA):

REFERENCE GROUP

Non diabetic
At Risk (Prediabetes)
Diagnosing Diabetes

GLYCOSYLATED HEMOGLOBIN (HBA1c) in %

<5.7

5.7 – 6.4

>= 6.5

Age > 19 Years

Goals of Therapy: < 7.0

Actions Suggested: >8.0

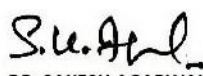
Age < 19 Years

Goal of therapy: <7.5

Therapeutic goals for glycemic control

REMARKS

- HbA1c is used for monitoring diabetic control. It reflects the mean plasma glucose over three months
 - HbA1c may be falsely low in diabetics with hemolytic disease. In these individuals a plasma fructosamine level may be used which evaluates diabetes over 15 days.
 - Inappropriately low HbA1c values may be reported due to hemolysis, recent blood transfusion, acute blood loss, hypertriglyceridemia, chronic liver disease. Drugs like dapsone, ribavirin, antiretroviral drugs, trimethoprim, may also cause interference with estimation of HbA1c, causing falsely low values.
 - HbA1c may be increased in patients with polycythemia or post-splenectomy.
 - Inappropriately higher values of HbA1c may be caused due to iron deficiency, vitamin B12 deficiency, alcohol intake, uremia, hyperbilirubinemia and large doses of aspirin.
 - Trends in HbA1c are a better indicator of diabetic control than a solitary test. 7. Any sample with >15% HbA1c should be suspected of having a hemoglobin variant, especially in a non-diabetic patient. Similarly, below 4% should prompt additional studies to determine the possible presence of variant hemoglobin.
 - HbA1c target in pregnancy is to attain level <6 % .
 - HbA1c target in paediatric age group is to attain level < 7.5 %.
- Method : Ion-exchange high-performance liquid chromatography (HPLC).
Reference : American Diabetes Associations. Standards of Medical Care in Diabetes 2015


DR. SAKESH AGARWAL
MBBS, DCP



SIN No:H6026393

Patient Name : Rajesh Kulkarni
5316220876
Age/Gender : 26/Male
Order Id : 5316220876
Referred By : Self
Customer Since : 18/Sept/2025
Sample Type : Flouride Plasma

Barcode : H6026393
Sample Collected On : 18/Sept/2025 07:00AM
Sample Received On : 18/Sept/2025 12:24PM
Report Generated On : 18/Sept/2025 01:00PM
Sample Temperature : Maintained ✓
Report Status : Final Report



DEPARTMENT OF BIOCHEMISTRY

Test Name	Value	Unit	Bio. Ref Interval
Fasting Blood Sugar			
Glucose, Fasting	98	mg/dl	70 - 100
Method: Hexokinase			

American Diabetes Association Reference Range :

Normal : < 100 mg/dl
Impaired fasting glucose(Prediabetes) : 100 - 126 mg/dl
Diabetes : >= 126 mg/dl

Conditions that can result in an elevated blood glucose level include: Acromegaly, Acute stress (response to trauma, heart attack, and stroke for instance), Chronic kidney disease, Cushing syndrome, Excessive consumption of food, Hyperthyroidism, Pancreatitis

A low level of glucose may indicate hypoglycemia, a condition characterized by a drop in blood glucose to a level where first it causes nervous system symptoms (sweating, palpitations, hunger, trembling, and anxiety), then begins to affect the brain (causing confusion, hallucinations, blurred vision, and sometimes even coma and death). A low blood glucose level (hypoglycemia) may be seen with: Adrenal insufficiency, Drinking excessive alcohol, Severe liver disease, Hypopituitarism, Hypothyroidism, Severe infections, Severe heart failure, Chronic kidney (renal) failure, Insulin overdose, Tumors that produce insulin (insulinomas), Starvation.

P. Bhatia

DR. PUNEETA BHATIA
MD, BIOCHEMISTRY
SENIOR CONSULTANT



Patient Name : Rajesh Kulkarni
5316220876
Age/Gender : 26/Male
Order Id : 5316220876
Referred By : Self
Customer Since : 18/Sept/2025
Sample Type : SERUM

Barcode : H6026393
Sample Collected On : 18/Sept/2025 07:00AM
Sample Received On : 18/Sept/2025 12:04PM
Report Generated On : 18/Sept/2025 02:20PM
Sample Temperature : Maintained ✓
Report Status : Final Report



DEPARTMENT OF BIOCHEMISTRY

Test Name	Value	Unit	Bio. Ref Interval
Lipid Profile			
Total Cholesterol Method: ENZymatic	150	mg/dl	Desirable : <200 Borderline: 200-239 High : >=240
Serum Triglycerides Method: GPO TRINDER	124	mg/dl	Desirable : <150 Borderline high : 150-199 High : 200-499 Very high : > 500
Serum HDL Cholesterol Method: Elimination/catalase	29.6	mg/dl	40 - 60
Serum LDL Cholesterol Method: Elimination/catalase	73.40	mg/dl	Optimal : <100 Near /Above Optimal:100 - 129 Borderline High:130 - 159 High : 160 - 189 Very High :>=190
Serum VLDL Cholesterol Method: Calculated	83.4	mg/dl	06 - 30
Total CHOL / HDL Cholesterol Ratio Method: Calculated	3.17	Ratio	3.30 - 4.40
LDL / HDL Cholesterol Ratio Method: Calculated	9.68	Ratio	Desirable/Low Risk: 0.5-3.0 Line/Moderate Risk: 3.0-6.0 Elevated/High Risk: >6.0
HDL / LDL Cholesterol Ratio Method: Calculated	0.51	Ratio	Optimal->0.4 Moderate-0.4 to 0.3 High-<0.3
Non-HDL Cholesterol Method: Calculated	117.4	mg/dl	0.0 - 160.0

Dyslipidemia is a disorder of fat or lipoprotein metabolism in the body and includes lipoprotein overproduction or deficiency. Dyslipidemias means increase in the level of one or more of the following:

Total Cholesterol, the "bad" cholesterol or low density lipoprotein (LDL) and/or triglyceride concentrations. Dyslipidemia also includes a decrease in the "good" cholesterol or high-density lipoprotein (HDL) concentration in the blood.

Lipid level assessments must be made following 9 to 12 hours of fasting, otherwise assay results might lead to erroneous interpretation.

Healthians labs report biological reference intervals (normal ranges) in accordance to the recommendations of The National Cholesterol Education Program (NCEP) & Adult Treatment Panel IV (ATP IV) guidelines providing the most desirable targets of various circulating lipid fractions in the blood. NCEP recommends that all adults above 20 years of age must be screened for abnormal lipid levels.

*NCEP recommends the assessment of 3 different samples drawn at intervals of 1 week for harmonizing biological variables that might be encountered in single

DR. WALIA MURSHIDA HUDA
MD, BIOCHEMISTRY



SIN No:H6026393



Patient Name : Rajesh Kulkarni
5316220876

Age/Gender : 26/Male

Order Id : 5316220876

Referred By : Self

Customer Since : 18/Sept/2025

Sample Type : SERUM

Barcode : H6026393



Sample Collected On : 18/Sept/2025 07:00AM

Sample Received On : 18/Sept/2025 12:04PM

Report Generated On : 18/Sept/2025 02:20PM

Sample Temperature : Maintained ✓

Report Status : Final Report

DEPARTMENT OF BIOCHEMISTRY

Test Name	Value	Unit	Bio. Ref Interval
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assays. Hence a single result of Lipid Profile may not be adequate for clinical decision making. Healthians' counselling team will reach you shortly to explain implications of your report. You may reach out to customer support helpline as well.

*High Triglyceride and low HDL levels are independent risk factors for Coronary Heart disease and requires further clinical consultation.

*Healthians lab performs direct LDL measurement which is more appropriate and may vary from other lab reports which provide calculated LDL values.

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SIN No:H6026393

Patient Name	: Rajesh Kulkarni	Barcode	: H6026393	
5316220876		Sample Collected On	: 18/Sept/2025 07:00AM	
Age/Gender	: 26/Male	Sample Received On	: 18/Sept/2025 12:04PM	
Order Id	: 5316220876	Report Generated On	: 18/Sept/2025 02:20PM	
Referred By	: Self	Sample Temperature	: Maintained ✓	
Customer Since	: 18/Sept/2025	Report Status	: Final Report	
Sample Type	: Serum			

DEPARTMENT OF BIOCHEMISTRY

Test Name	Value	Unit	Bio. Ref Interval
Liver Function Test (LFT)			
Serum Bilirubin, (Total)	0.29	mg/dl	0.3 - 1.2
Method: Vanadate oxidation			
Serum Bilirubin, (Direct)	0.07	mg/dl	0 - 0.3
Method: Vanadate oxidation			
Serum Bilirubin, (Indirect)	0.22	mg/dl	0.0 - 0.8
Method: Calculated			
Aspartate Aminotransferase (AST/SGOT)	19.00	U/L	5 - 34
Method: Modified IFCC			
Alanine Aminotransferase (ALT/SGPT)	11.00	U/l	10 - 49
Method: Modified IFCC			
Alkaline Phosphatase (ALP)	140.00	U/L	38 - 126
Method: DEA BUFFER			
Gamma Glutamyl Transferase (GGT)	5.0	U/L	5-38.0
Method: IFCC			
Serum Total Protein	7.10	g/dl	5.7-8.2
Method: Biuret			
Serum Albumin	4.60	g/dl	3.4 - 4.8
Method: Bromo Cresol Green(BCG)			
Serum Globulin	2.50	gm/dl	3.0 - 4.2
Method: Calculated			
Albumin/Globulin Ratio	1.84	Ratio	1.2 - 2.5
Method: Calculated			
SGOT/SGPT Ratio	1.73	Ratio	0.7 - 1.4
Method: Calculated			

Bilirubin is a yellowish pigment found in bile and is a breakdown product of normal heme catabolism. Elevated levels results from increased bilirubin production (eg hemolysis and ineffective erythropoiesis); decreased bilirubin excretion (eg; obstruction and hepatitis); and abnormal bilirubin metabolism (eg; hereditary and neonatal jaundice). Conjugated (direct) bilirubin is elevated more than unconjugated (indirect) bilirubin in viral hepatitis; drug reactions, alcoholic liver disease conjugated (direct) bilirubin is also elevated more than unconjugated (indirect) bilirubin when there is some kind of blockage of the bile ducts like in Gallstones getting into the bile ducts tumors & Scarring of the bile ducts. Increased unconjugated (indirect) bilirubin may be a result of hemolytic or pernicious anemia, transfusion reaction & a common metabolic condition termed Gilbert syndrome.

AST levels increase in viral hepatitis, blockage of the bile duct ,cirrhosis of the liver, liver cancer, kidney failure, hemolytic anemia, pancreatitis, hemochromatosis. Ast levels may also increase after a heart attck or strenuous activity. ALT is commonly measured as a part of a diagnostic evaluation of hepatocellular injury, to determine liver health. Elevated ALP levels are seen in Biliary Obstruction, Osteoblastic Bone Tumors, Osteomalacia, Hepatitis, Hyperparathyroidism, Leukemia, Lymphoma, paget's disease, Rickets, Sarcoidosis etc.

Elevated serum GGT activity can be found in diseases of the liver, Biliary system and pancreas. Conditions that increase serum GGT are obstructive liver disease, high alcohol consumption and use of enzyme-including drugs etc.



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SIN No:H6026393

The test was performed by **Healthians Labs (A Unit of Expedient Healthcare Marketing Pvt. Ltd.) - Plot 1 & 2, Udyog Vihar Phase 4, Gurgaon - 122016** and validated by Authorized Medical Practitioner / Lab Doctor

Patient Name : Rajesh Kulkarni
5316220876
Age/Gender : 26/Male
Order Id : 5316220876
Referred By : Self
Customer Since : 18/Sept/2025
Sample Type : Serum

Barcode : H6026393
Sample Collected On : 18/Sept/2025 07:00AM
Sample Received On : 18/Sept/2025 12:04PM
Report Generated On : 18/Sept/2025 02:20PM
Sample Temperature : Maintained ✓
Report Status : Final Report



DEPARTMENT OF BIOCHEMISTRY

Test Name	Value	Unit	Bio. Ref Interval
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Serum total protein, also known as total protein, is a biochemical test for measuring the total amount of protein in serum..Protein in the plasma is made up of albumin and globulin. Higher-than-normal levels may be due to: Chronic inflammation or infection, including HIV and hepatitis B or C, Multiple myeloma,Waldenstrom's disease. Lower-than-normal levels may be due to: Agammaglobulinemia, Bleeding (hemorrhage), Burns, Glomerulonephritis, Liver disease, Malabsorption, Malnutrition, Nephrotic - Human serum albumin is the most abundant protein in human blood plasma. It is produced in the liver.Albumin constitutes about half of the blood serum protein. Low blood albumin levels (hypoalbuminemia) can be caused by: Liver disease like cirrhosis of the liver, nephrotic syndrome, protein-losing enteropathy, Burns, hemodilution, increased vascular permeability or decreased lymphatic clearance, malnutrition and wasting etc.

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SIN No:H6026393



Patient Name : Rajesh Kulkarni
5316220876
Age/Gender : 26/Male
Order Id : 5316220876
Referred By : Self
Customer Since : 18/Sept/2025
Sample Type : SERUM

Barcode : H6026393
Sample Collected On : 18/Sept/2025 07:00AM
Sample Received On : 18/Sept/2025 12:04PM
Report Generated On : 18/Sept/2025 02:20PM
Sample Temperature : Maintained ✓
Report Status : Final Report



DEPARTMENT OF BIOCHEMISTRY IRON STUDY

Test Name	Value	Unit	Bio. Ref Interval
Iron study			
Serum Iron Method: Ferrozine	16.4	ug/dl	50-170
UIBC Method: Nitroso-PSAP	374.10	ug/dl	120- 470
Serum Total Iron Binding Capicity (TIBC) Method: FE+UIBC (saturation with iron)	390.5	µg/dl	250 - 400
Transferrin Saturation % Method: Calculated	4.2	%	15 - 50

Iron participates in a variety of vital processes in the body varying from cellular oxidative mechanisms to the transport and delivery of oxygen to body cells. It is a constituent of the oxygen-carrying chromoproteins, haemoglobin and myoglobin, as well as various enzymes, such as cytochrome oxidase and peroxidases. Serum iron may be increased in hemolytic, megaloblastic and aplastic anemias, and in hemochromatosis acute leukemia, lead poisoning, pyridoxine deficiency, thalassemia, excessive iron therapy, and after repeated transfusions. Drugs causing increased serum iron include chloramphenicol, cisplatin, estrogens (including oral contraceptives), ethanol, iron dextran, and methotrexate. Iron can be decreased in iron-deficiency anemia, acute and chronic infections, carcinoma, nephrotic syndrome hypothyroidism, in protein- calorie malnutrition, and after surgery.

Transferrin is the primary plasma iron transport protein, which binds iron strongly at physiological pH. Transferrin is generally only 25% to 30% saturated with iron. The additional amount of iron that can be bound is the unsaturated iron-binding capacity (UIBC). Diurnal variation is seen in serum iron levels-normal values in midmorning, low values in midafternoon, very low values (approximately 10 µg/dL) near midnight.

TIBC measures the blood's capacity to bind iron with transferrin (TRF). Estrogens and oral contraceptives increase TIBC levels. Asparaginase, chloramphenicol, corticotropin, cortisone, and testosterone decrease the TIBC levels.

% saturation represents the amount of iron-binding sites that are occupied. Iron saturation is a better index of iron stores than serum iron alone. % saturation is decreased in iron deficiency anemia (usually <10% in established deficiency).

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SIN No:H6026393



Patient Name : Rajesh Kulkarni
5316220876
Age/Gender : 26/Male
Order Id : 5316220876
Referred By : Self
Customer Since : 18/Sept/2025
Sample Type : SERUM

Barcode : H6026393
Sample Collected On : 18/Sept/2025 07:00AM
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Report Generated On : 18/Sept/2025 02:20PM
Sample Temperature : Maintained ✓
Report Status : Final Report



DEPARTMENT OF BIOCHEMISTRY

Test Name	Value	Unit	Bio. Ref Interval
Kidney Function Test1 (KFT1)			
Serum Creatinine Method: Jaffes Kinetic	0.73	mg/dl	0.5-1.1
Serum Uric Acid Method: Uricase/Peroxidase	6.5	mg/dl	2.6 - 6
Serum Calcium Method: Arsenazo III	9.8	mg/dl	8.7-10.4
Serum Phosphorus Method: Phosphomolybdate/UV	4.6	mg/dl	2.4-5.1
Serum Sodium Method: ISE (Indirect)	139	mmol/L	132 - 146
Serum Chloride Method: ISE (Indirect)	110	mmol/L	99-109
Blood Urea Method: Urease	35	mg/dl	19.3-49.38
Blood Urea Nitrogen (BUN) Method: Calculated	16.4	mg/dl	8-20
Bun/Creatinine Ratio Method: Calculated	22.41	Ratio	
Urea/Creatinine Ratio	47.95		

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SIN No:H6026393



Patient Name : Rajesh Kulkarni
5316220876
Age/Gender : 26/Male
Order Id : 5316220876
Referred By : Self
Customer Since : 18/Sept/2025
Sample Type : URINE

Barcode : H6026393
Sample Collected On : 18/Sept/2025 07:00AM
Sample Received On : 18/Sept/2025 12:45PM
Report Generated On : 18/Sept/2025 03:34PM
Sample Temperature : Maintained ✓
Report Status : Final Report



DEPARTMENT OF CLINICAL PATHOLOGY

Test Name	Value	Unit	Bio. Ref Interval
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Urine Routine & Microscopy Extended

PHYSICAL EXAMINATION

Colour	Pale Yellow		Pale Yellow
Method: Visual			
Volume	15.00	mL	
Method: Visual			
Appearance	Slightly Hazy		Clear
Method: Visual			

CHEMICAL EXAMINATION

Specific Gravity	1.020		1.001 - 1.035
Method: Polyelectrolyte dissociation			
pH	5.0		4.5 - 7.5
Method: acid-base indicator method			
Glucose	Negative		Negative
Method: enzymic method			
Urine Protein	Negative		Negative
Method: protein error method			
Ketones	Negative		Negative
Method: Sodium nitrosoferricyanide			
Urobilinogen	Normal		Normal
Method: Diazonium salt			
Bilirubin	Negative		Negative
Method: Azo reaction			
Nitrite	Negative		Negative
Method: Griess method			
Blood	Negative		Negative
Method: Peroxidase-like method			
Leucocyte Esterase	2+		Negative
Method: diazonium salt			

MICROSCOPIC EXAMINATION

Pus Cells	15-20	/HPF	0 - 5
Method: Flow micro Imaging			
Epithelial cells	4-5	/HPF	0 - 5
Method: Flow micro Imaging			
RBCs	Nil	/HPF	Nil

S. U. Agarwal
DR. SAKESH AGARWAL
MBBS, DCP



SIN No:H6026393



Patient Name : Rajesh Kulkarni
5316220876
Age/Gender : 26/Male
Order Id : 5316220876
Referred By : Self
Customer Since : 18/Sept/2025
Sample Type : URINE

Barcode : H6026393
Sample Collected On : 18/Sept/2025 07:00AM
Sample Received On : 18/Sept/2025 12:45PM
Report Generated On : 18/Sept/2025 03:34PM
Sample Temperature : Maintained ✓
Report Status : Final Report



DEPARTMENT OF CLINICAL PATHOLOGY

Test Name	Value	Unit	Bio. Ref Interval
Method: Flow micro Imaging			
Casts	Nil		Nil
Method: Flow micro Imaging			
Crystals	Nil		Nil
Method: Flow micro Imaging			
Bacteria	Absent		Absent
Method: Flow micro Imaging			
Yeast Cell	Absent		Absent
Method: Flow micro Imaging			
Others (Non Specific)	Nil		
Method: Microscopic Examination			

The main indication for testing for glucose in urine is detection of unsuspected diabetes mellitus or follow-up of known diabetic patients. Renal glycosuria accounts for 5% of cases of glycosuria in general population.

Proteinuria can be seen in nephrotic syndrome, pyelonephritis, heavy metal poisoning, tuberculosis of kidney, interstitial nephritis, cystinosis, Fanconi syndrome, rejection of kidney transplant. Hemodynamic proteinuria is transient and can be seen in high fever, hypertension, heavy exercise, congestive cardiac failure, seizures, and exposure to cold. Post-renal proteinuria is caused by inflammatory or neoplastic conditions in renal pelvis, ureter, bladder, prostate, or urethra.

Ketonuria can be seen in uncontrolled Diabetes mellitus with ketoacidosis, Glycogen storage disorder, starvation, persistent vomiting in children, weight reduction program, fever in children, severe thyrotoxicosis, pregnancy and protein calorie malnutrition.

Presence of bilirubin in urine indicates conjugated hyperbilirubinemia (obstructive or hepatocellular jaundice). Bile salts along with bilirubin can be detected in urine in cases of obstructive jaundice. Normally about 0.5-4 mg of urobilinogen is excreted in urine in 24 hours. Therefore, a small amount of urobilinogen is normally detectable in urine. Increased urobilinogen in urine can be seen due to hemolysis, megaloblastic anemia and haemorrhage in tissues. Decreased urobilinogen can be seen in obstructive jaundice, reduction of intestinal bacterial flora, neonates and following antibiotic treatment. The presence of abnormal number of intact red blood cells in urine is called as hematuria. It implies presence of a bleeding lesion in the urinary tract. Hematuria can be seen in glomerular diseases like Glomerulonephritis, Berger's disease, lupus nephritis, Henoch-Schonlein purpura, non glomerular diseases like Calculus, tumor, infection, tuberculosis, pyelonephritis, hydronephrosis, polycystic kidney disease, trauma, after strenuous physical exercise, diseases of prostate (benign hyperplasia of prostate, carcinoma of prostate).

Nitrites are not present in normal urine. Ingested nitrites are converted to nitrate and excreted

in urine. If gram-negative bacteria (e.g. E.coli, Salmonella, Proteus, Klebsiella, etc.) are present in urine, they will reduce the nitrates to nitrites through the action of bacterial enzyme nitrate reductase. As E. coli is the commonest organism causing urinary tract infection, this test is helpful as a screening test for urinary tract infection.

Some organisms like Staphylococci or Pseudomonas do not reduce nitrate to nitrite and therefore in such infections nitrite test is negative.

Leucocyte esterase test detects esterase enzyme released in urine from granules of leucocytes. Thus the test is positive in pyuria.

DR. SAKESH AGARWAL
MBBS, DCP



SIN No:H6026393

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MC-4245

Patient Name : Rajesh Kulkarni
5316220876
Age/Gender : 26/Male
Order Id : 5316220876
Referred By : Self
Customer Since : 18/Sept/2025
~~Sample Type~~ : ~~Whole Blood EDTA~~

Barcode : H6026393
Sample Collected On : 18/Sept/2025 07:00AM
Sample Received On : 18/Sept/2025 12:21PM
Report Generated On : 18/Sept/2025 02:19PM
Sample Temperature : Maintained ✓
Report Status : Final Report



DEPARTMENT OF HAEMATOLOGY

Test Name	Value	Unit	Bio. Ref Interval
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Complete Haemogram

Haemoglobin (HB) Method: Photometric Measurement	11.2	g/dl	12.0-15.0
Total Leucocyte Count (TLC) Method: Coulter Principle	7.0	10 ³ /uL	4.0-10.0
Hematocrit (PCV) Method: Calculated	35	%	36.0-46.0
Red Blood Cell Count (RBC) Method: Coulter Principle	4.50	millions/cumm	3.80-4.80
Mean Corp Volume (MCV) Method: Derived from RBC Histogram	77.8	FL	83.0-101.0
Mean Corp Hb (MCH) Method: Calculated	24.9	pg	24.0-30.0
Mean Corp Hb Conc (MCHC) Method: Calculated	32.0	gm%	31.5-34.5
RDW - CV Method: Derived from RBC Histogram	14.7	%	12.3-14.0
RDW - SD Method: Derived from RBC Histogram	40.30	FL	39.0-46.0
Mentzer Index Method: Calculated	17.29	Ratio	
RDWI Method: Calculated	254.15	Ratio	
Green and king index Method: Calculated	79	Ratio	

Differential Leucocyte Count

Neutrophils Method: VCSn Technology	61.5	%	40 - 75
Lymphocytes Method: VCSn Technology	28.2	%	20 - 45
Monocytes Method: VCSn Technology	5.6	%	01 - 10
Eosinophils Method: VCSn Technology	4.1	%	01 - 06
Basophils Method: VCSn Technology	0.6	%	00 - 02

DR. SAKESH AGARWAL
MBBS, DCP



SIN No:H6026393

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MC-4245

Patient Name	: Rajesh Kulkarni	Barcode	: H6026393	
5316220876		Sample Collected On	: 18/Sept/2025 07:00AM	
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Referred By	: Self	Sample Temperature	: Maintained ✓	
Customer Since	: 18/Sept/2025	Report Status	: Final Report	
Sample Type	: Whole Blood EDTA			

DEPARTMENT OF HAEMATOLOGY

Test Name	Value	Unit	Bio. Ref Interval
Absolute Leucocyte Count			
Absolute Neutrophil Count (ANC)	4.31	10 ³ /uL	2.0-7.0
Method: Calculated			
Absolute Lymphocyte Count (ALC)	1.97	10 ³ /uL	1.0-3.0
Method: Calculated			
Absolute Monocyte Count	0.39	10 ³ /uL	0.2-1.0
Method: Calculated			
Absolute Eosinophil Count (AEC)	0.29	10 ³ /uL	0.02-0.5
Method: Calculated			
Absolute Basophil Count	0.04	10 ³ /uL	0.0 - 0.10
Method: Calculated			
Platelet Count(PLT)	178	10 ³ /μl	150-410
Method: Coulter Principle			
MPV	12.4	FL	7.9-9.2
Method: Derived from PLT Histogram			
ESR	28	mm/1st hr.	0 - 19
Method: Kinetic Red Cell Aggregation			

The International Council for Standardization in Haematology (ICSH) recommends reporting of absolute counts of various WBC subsets for clinical decision making. This test has been performed on a fully automated 5 part differential cell counter which counts over 10,000 WBCs to derive differential counts. A complete blood count is a blood panel that gives information about the cells in a patient's blood, such as the cell count for each cell type and the concentrations of Hemoglobin and platelets. The cells that circulate in the bloodstream are generally divided into three types: white blood cells (leukocytes), red blood cells (erythrocytes), and platelets (thrombocytes). Abnormally high or low counts may be physiological or may indicate disease conditions, and hence need to be interpreted clinically.

The Mentzer index is used to differentiate iron deficiency anaemia beta thalassemia trait. If a CBC indicates microcytic anaemia, these are two of the most likely causes, making it necessary to distinguish between them.

If the quotient of the mean corpuscular volume divided by the red blood cell count is then 13, thalassemia is more likely. If the result is greater than 13, then iron-deficiency anaemia is more likely. Green and King Index used to differentiate IDA from thalassemia trait value >65 is likely to be Iron Deficiency Anemia and value <65 Beta Thalassemia Trait. For RDWI Value >220 more likely to be Iron Deficiency Anemia and value <220 more likely to be Beta Thalassemia Trait.

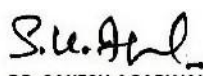
ESR is a non-specific phenomenon, its measurement is clinically useful in disorders associated with an increased production of acute-phase proteins. It provides an index of progress of the disease in rheumatoid arthritis or tuberculosis, and it is of considerable value in diagnosis of temporal arteritis and polymyalgia rheumatica. It is often used if multiple myeloma is suspected, but when the myeloma is non-secretory or light chain, a normal ESR does not exclude this diagnosis.

An elevated ESR occurs as an early feature in myocardial infarction. Although a normal ESR cannot be taken to exclude the presence of organic disease, the vast majority of acute or chronic infections and most neoplastic and degenerative diseases are associated with changes in the plasma proteins that increased ESR values.

An increased ESR in subjects who are HIV seropositive seems to be an early predictive marker of progression toward acquired immune deficiency syndrome (AIDS).

The ESR is influenced by age, stage of the menstrual cycle and medications taken (corticosteroids, contraceptive pills). It is especially low (0–1 mm) in polycythaemia, hypofibrinogenaemia and congestive cardiac failure and when there are abnormalities of the red cells such as poikilocytosis, spherocytosis, or sickle cells.

In cases of performance enhancing drug intake by athletes the ESR values are generally lower than the usual value for the individual and as a result of the increase in haemoglobin (i.e. the effect of secondary polycythaemia).


DR. SAKESH AGARWAL
MBBS, DCP



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Patient Name : Rajesh Kulkarni
5316220876
Age/Gender : 26/Male
Order Id : 5316220876
Referred By : Self
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Sample Type : Whole Blood EDTA

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Sample Temperature : Maintained ✓
Report Status : Final Report

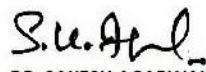


DEPARTMENT OF HAEMATOLOGY

Test Name	Value	Unit	Bio. Ref Interval
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SIN No:H6026393


DR. SAKESH AGARWAL
MBBS, DCP

Patient Name	: Rajesh Kulkarni	Barcode	: H6026393	
5316220876		Sample Collected On	: 18/Sept/2025 07:00AM	
Age/Gender	: 26/Male	Sample Received On	: 18/Sept/2025 12:04PM	
Order Id	: 5316220876	Report Generated On	: 18/Sept/2025 01:26PM	
Referred By	: Self	Sample Temperature	: Maintained ✓	
Customer Since	: 18/Sept/2025	Report Status	: Final Report	
Sample Type	: Serum			

DEPARTMENT OF IMMUNOLOGY

Test Name	Value	Unit	Bio. Ref Interval
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Vitamin B12

VITAMIN B12	503	pg/ml	211 - 912
Method: CLIA			

Vitamin B12 is a coenzyme that is involved in two very important metabolic functions vital to normal cell growth and DNA synthesis: 1) the synthesis of methionine, and 2) the conversion of methylmalonyl CoA to succinyl CoA. Deficiency of this vitamin can lead to megaloblastic anemia and ultimately to severe neurological problems. Also causes macrocytic anemia, glossitis, peripheral neuropathy, weakness, hyperreflexia, ataxia, loss of proprioception, poor coordination, and affective behavioral changes. A significant increase in RBC MCV may be an important indicator of vitamin B12 deficiency.

Patients taking vitamin B12 supplementation may have misleading results. A normal serum concentration of B12 does not rule out tissue deficiency of vitamin B12 . The most sensitive test for B12 deficiency at the cellular level is the assay for MMA. If clinical symptoms suggest deficiency, measurement of MMA and homocysteine should be considered, even if serum B12 concentrations are normal.

P. Bhatia

DR. PUNEETA BHATIA
MD, BIOCHEMISTRY
SENIOR CONSULTANT



Patient Name	: Rajesh Kulkarni	Barcode	: H6026393	
5316220876		Sample Collected On	: 18/Sept/2025 07:00AM	
Age/Gender	: 26/Male	Sample Received On	: 18/Sept/2025 12:04PM	
Order Id	: 5316220876	Report Generated On	: 18/Sept/2025 02:11PM	
Referred By	: Self	Sample Temperature	: Maintained ✓	
Customer Since	: 18/Sept/2025	Report Status	: Final Report	
Sample Type	: Serum			

DEPARTMENT OF IMMUNOLOGY

Test Name	Value	Unit	Bio. Ref Interval
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Vitamin D, 25-Hydroxy

VITAMIN D (25 - OH VITAMIN D)	11.37	ng/ml	30 - 100
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Method: CLIA

VITAMIN D STATUS	VITAMIN D 25 HYDROXY (ng/mL), Adult	VITAMIN D 25 HYDROXY (ng/mL), Pediatric
DEFICIENCY	<20	<15
INSUFFICIENCY	20 - 30	15 - 20
SUFFICIENCY	30 - 100	20 - 100

Vitamin D is a lipid-soluble steroid hormone that is produced in the skin through the action of sunlight or is obtained from dietary sources. The role of vitamin D in maintaining homeostasis of calcium and phosphorus is well established.

The assay measures both D2 (Ergocalciferol) and D3 (Cholecalciferol) metabolites of vitamin D. Vitamin D status is best determined by measurement of 25 hydroxy vitamin D, as it is the major circulating form and has longer half life (2-3 weeks) than 1,25 Dihydroxy vitamin D (5-8 hrs)

The reference ranges discussed in the preceding are related to total 25-OHD; as long as the combined total is 30 ng/mL or more, the patient has sufficient vitamin D. Levels needed to prevent rickets and osteomalacia (15 ng/mL) are lower than those that dramatically suppress parathyroid hormone levels (20-30 ng/mL). In turn, those levels are lower than levels needed to optimize intestinal calcium absorption (34 ng/mL). Neuromuscular peak performance is associated with levels approximately 38 ng/mL.



DR. WALIA MURSHIDA HUDA
MD, BIOCHEMISTRY



SIN No:H6026393

Patient Name	: Rajesh Kulkarni	Barcode	: H6026393	
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Order Id	: 5316220876	Report Generated On	: 18/Sept/2025 01:26PM	
Referred By	: Self	Sample Temperature	: Maintained ✓	
Customer Since	: 18/Sept/2025	Report Status	: Final Report	
Sample Type	: Serum			

DEPARTMENT OF IMMUNOLOGY

Test Name	Value	Unit	Bio. Ref Interval
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Thyroid Profile (Total T3, T4, TSH)

Tri-Iodothyronine (T3, Total)	1.00	ng/ml	0.60-1.81
Method: CLIA			
Thyroxine (T4, Total)	9.20	ug/dl	3.2-12.6
Method: CLIA			
Thyroid Stimulating Hormone (TSH)-Ultrasensitive	3.2060	μIU/ml	0.55-4.78
Method: CLIA			

Pregnancy interval	Bio Ref Range for TSH in uIU/ml (As per American Thyroid Association)
First trimester	0.1 - 2.5
Second trimester	0.2 - 3.0
Third trimester	0.3 - 3.0

Healthians recommends that the following potential sources of variation should be considered while interpreting thyroid hormone results:

1. Thyroid hormones undergo rhythmic variation within the body this is called circadian variation in TSH secretion: Peak levels are seen between 2-4 AM. Minimum levels seen between 6-10 AM. This variation may be as much as 50% thus, influence of sampling time needs to be considered for clinical interpretation.
2. Circulating forms of T3 and T4 are mostly reversibly bound with Thyroxine binding globulins (TBG), and to a lesser extent with albumin and Thyroid binding Pre-Albumin. Thus the conditions in which TBG and protein levels alter such as chronic liver disorders, pregnancy, excess of estrogens, androgens, anabolic steroids and glucocorticoids may cause misleading total T3, total T4 and TSH interpretations.
3. Total T3 and T4 levels are seen to have physiological rise during pregnancy and in patients on steroid treatment.
4. T4 may be normal even in the presence of hyperthyroidism under the following conditions : T3 thyrotoxicosis, Hypoproteinemia related reduced binding, during intake of certain drugs (eg Phenytoin, Salicylates etc)
5. Neonates and infants have higher levels of T4 due to increased concentration of TBG
6. TSH levels may be normal in central hypothyroidism, recent rapid correction of hypothyroidism or hyperthyroidism, pregnancy, phenytoin therapy etc.
7. TSH values of <0.03 uIU/mL must be clinically correlated to evaluate the presence of a rare TSH variant in certain individuals which is undetectable by conventional methods.
8. Presence of Autoimmune disorders may lead to spurious results of thyroid hormones
9. Various drugs can lead to interference in test results.
10. Healthians recommends evaluation of unbound fractions, that is free T3 (fT3) and free T4 (fT4) for clinic-pathologic correlation, as these are the metabolically active forms.

*** End Of Report ***

P. Bhatia

DR. PUNEETA BHATIA
MD, BIOCHEMISTRY
SENIOR CONSULTANT



Terms & Conditions:

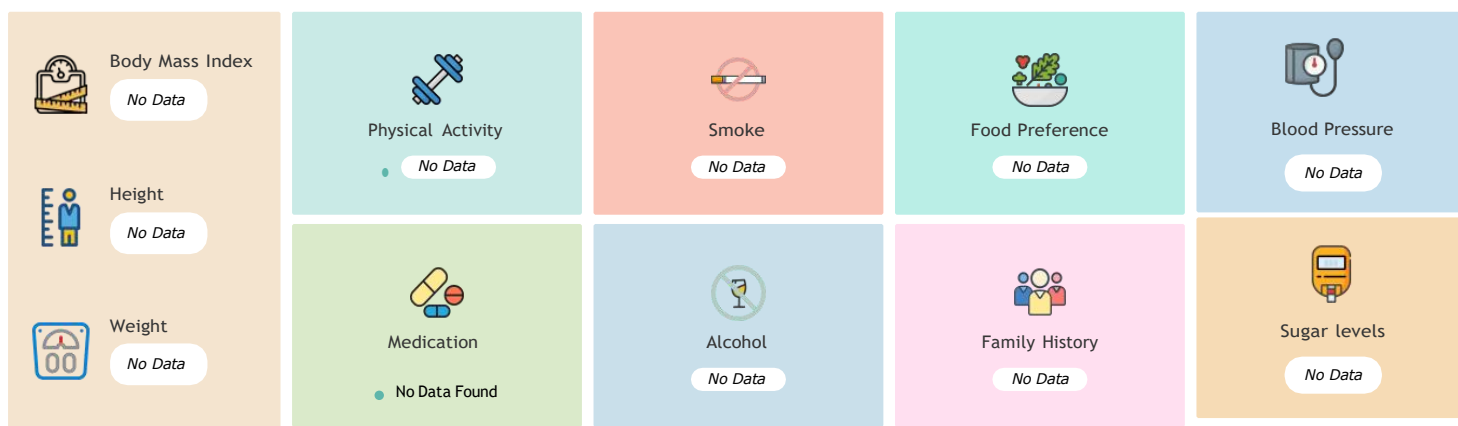
- 1) Machine Data is available for last 7 days only. In case of manual testing & outsourced testing, machine data will not be available.
- 2) CBC parameters may vary when it is manually reviewed by the Pathologists.
- 3) **For Thyroid tests** - Circulating TSH shows a normal circadian rhythm with a peak between 11pm-5am and a nadir between 5pm-8pm. TSH values are also lowered after food when compared to fasting in a statistically significant manner. This variation is of the order of $\pm 50\%$, hence time of day and fasting status have influence on the reported TSH level.
- 4) **For Lipid profile** - Lipid and Lipoprotein concentrations vary during the normal course of daily activity. Also, certain drugs, diet and alcohol can have lasting effects on Triglyceride levels. To obtain best results for Lipid testing, a strict fasting of 10-12 hours with a light meal on the previous night is recommended.
- 5) Test results released pertain to the specimen submitted.
- 6) Test results are dependent on the quality of the sample received by the Lab.
- 7) The tests are carried out in the lab with the presumption that the specimen belongs to the patient named or identified in the bill/test request form/booking ID.
- 8) The reported results are for information and are subject to confirmation and interpretation by the referring doctor to co-relate clinically.
- 9) Test results may show interlaboratory variations.
- 10) Liability of Healthians for deficiency of services or other errors and omissions shall be limited to the fee paid by the patient for the relevant laboratory services.
- 11) This report is not subject to use for any medico-legal purposes.
- 12) Few of the tests might be outsourced to partner labs as and when required.

ADVISORY

Health Advisory

Rajesh Kulkarni

Booking ID : 5316220876



SUGGESTED NUTRITION

SUGGESTED NUTRITION

Do's

- Include seeds like flaxseeds, chia seeds, sunflower seeds
- Include fruits like apples, berries and melons in your diet
- Have a balanced diet that includes whole grains, pulses, dairy, fruits, vegetables, nuts and healthy fats
- Have dates and figs
- Take vitamin C rich foods like citrus fruits, strawberries and green, leafy vegetables
- Include calcium rich foods like milk, yoghurt, cheese and green, leafy vegetables
- Include Brazil nuts, sesame seeds, sunflower seeds
- Take fresh fruit and vegetable juices

Dont's

- Limit sugar intake
- Limit tea and coffee
- Decrease intake of colas and sugary drinks
- Avoid refined carbs, processed foods
- Avoid the use of oil and avoid sauces and dressings
- Limit intake of salt
- Reduce caffeine intake
- Avoid flavoured and seasoned foods
- Avoid saturated fats, trans fats, oily and greasy foods like cakes, creamy or fried foods
- Avoid red meat and organ meats

SUGGESTED LIFESTYLE

SUGGESTED LIFESTYLE

Do's

- Lose weight gradually and stay active
- Maintain ideal weight
- Have regular exposure to sunlight

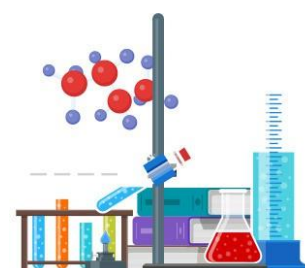
Dont's

- Avoid late night heavy meals
- Avoid overworking or being stressed for long time
- Avoid smoking and alcohol
- Avoid overexertion without having food or drink
- Avoid strenuous exercises
- Don't ignore your body signals and don't skip your regular health check-ups
- Limit dining out

SUGGESTED FUTURE TESTS

SUGGESTED FUTURE TESTS

- Glycated Hemoglobin (HbA1c) - Every 3 Month
- Blood Glucose Fasting - Every 1 Week
- Glucose Postprandial - Every 1 Week
- Liver Function Test - Every 3 Month
- Kidney Function Test - Every 3 Month
- Lipid Profile - Every 3 Month
- Microalbumin, Urine spot - Every 6 Month



HEALTH ADVISORY

Suggestions for Health & Well-being

Rajesh Kulkarni

Booking ID : 5316220876

PHYSICAL ACTIVITY

PHYSICAL ACTIVITY

Physical activities can vary from Regular walks (Brisk or normal), Jogging , Sports, Stretching, Yoga to light weight lifting etc. It is recommended to partake in physical activity at least 30 minutes a day for 3-4 days a week.

If regular workout is difficult, then we can adapt changes such as using stairs instead of lift/escalators and doing household work!



BALANCED DIET

A balanced diet is the key to healthy lifestyle. Include Whole grains, vegetables, whole fruits, nuts, seeds, beans, plant oils in your diet.

It is recommended to always have a high protein breakfast and a light dinner. Avoid items such as processed foods, potatoes and high calorie/sugar products. Don't forget to drink water regularly!

BALANCED DIET

STRESS MANAGEMENT

STRESS MANAGEMENT

Managing stress is an essential part of well-being. Some day to day changes can help such as having sufficient sleep (6-8 hours), indulging yourself in meditation, positive attitude towards lifestyle, using humor, traveling, talking to people whom you feel comfortable with and making time for hobbies by doing what you love to do.



BMI

BMI recommended range is 18.5 to 24.9. Your BMI is 29.35, which is on a higher side.

BMI INFORMATION NOT AVAILABLE
Please fill your Health Karma to know your BMI results and BMI for your body helps prevent many untimely diseases and goes a long way.

BMI CHART



BMI

Your test report has indicated that you have certain deficiencies in your body which may hamper your health & wellbeing in the longer run.

In order to fulfill the gaps in nutrition and promote a healthier body we suggest you the following supplements mentioned below:

Deficiency/Out of Range Parameter(s)	Suggested Supplement
Glycated Hemoglobin (HbA1c)	DIABEAT-EASE
Bilirubin Total, Serum	LIV-UP

To order, call 1800-572-000-4

Suggestions for Improving Deficiencies



DIABEAT-EASE

Manage diabetes the all-natural way!

An all-natural supplement that helps in lowering your blood sugar levels, thus preventing the onset of diabetes and managing it if you are already a diabetic. By reducing blood sugar levels, this naturally-sourced diabetes supplement enables you to lead a productive life, while managing your diabetes in a safe and natural way.

Remember, ignoring diabetes can cause a lot of serious complications, including:

• Vision & Hearing Loss | • Nerve Damage | • Heart Attack | • Stroke | • Dementia

Infused with the ages-proven goodness of all-natural ingredients, DIABEAT-EASE is the perfect supplement to help you control diabetes without having to worry about side-effects. Sourced from nature's own pharmacy of herbs, the ingredients in DIABEAT-EASE present the following benefits:

Saunf

Helps control blood pressure & manage diabetes

Karela

Reduces blood sugar & reduces cholesterol levels

Chirata

Helps manage high blood pressure, diabetes & detoxifies blood

Ashwagandha

Reduces blood sugar, cholesterol, & triglycerides levels

Vijayasar

Helps manage diabetes by lowering down sugar cravings



LIV-UP

De-toxify your body with a healthier liver.

LIV-UP is a scientifically formulated and clinically proven all-natural supplement that takes care of your liver and its functions. This ayurvedic supplement keeps your liver cool, and optimally functioning, thus promoting healthy digestion. Take the all-natural road to robust liver health with LIV-UP.

Untreated or unmanaged liver issues can cause grave and even lethal complications, which include:

• Liver Infections | • Liver Cancer | • Liver Failure | • Elevated Blood Toxin Levels | • Liver Cirrhosis

Infused with the ages-proven goodness of all-natural ingredients, LIV-UP is the perfect supplement to promote and maintain good liver health, without having to worry about side effects. Sourced from nature's own pharmacy of herbs, the ingredients in LIV-UP present the following benefits:

Methi Dana

Reducing the risk of developing fatty liver disease.

Triphala

Promotes liver function & boosts immunity

Ginger

Supports liver health & prevents liver inflammation

Yellow Myrobalan

Keeps the liver cool & promotes optimal functioning

Kulki

Reduces inflammation & protects the liver from injury

Deficiency/Out of Range Parameter(s)	Suggested Supplement
Iron, Serum	IRON POWER
Vitamin D Total-25 Hydroxy	VITAMIN D3

To order, call 1800-572-000-4

Suggestions for Improving Deficiencies



IRON POWER

Your all natural weapon against Anaemia

IRON-POWER is a scientifically formulated and clinically proven all-natural supplement that helps you replenish iron in your system and significantly reduce the risk of anemia. This ayurvedic supplement increases blood production and iron levels to keep your body functioning optimally. Keep overall health robust and treat iron deficiency with IRON-POWER

It's widely known that iron is crucial for your health. A lack of iron intake or iron deficiency can cause serious health issues, such as:

- Heart problems
- Growth issues in children
- Pregnancy complications
- Depression
- Increased infection risk

Infused with the ages-proven goodness of all-natural ingredients, IRON-POWER is the perfect supplement to enhance and maintain your iron levels without having to worry about side effects. Sourced from nature's own pharmacy of herbs, the ingredients in IRON-POWER present the following benefits:

Orange Peel Extract

Rich in vitamin C. Helps maintain iron levels



VITAMIN D3

Improve bone health with enhanced calcium absorption, the natural way

Make your muscles and bones stronger with VITAMIN D3. Sourced from natural substances, it helps in regulating the absorption of calcium and phosphorus, which help keep your bones strong and enhancing the normal immune system functioning. Vitamin D3 is an essential nutrient that's critical for normal growth and development of bones and teeth, as well as improved resistance against certain diseases.

Remember, a lack of vitamin D3 can cause dangerous health situations.

- Rickets (in children)
- Brittle Bones
- Osteoporosis
- Weakened Bones (in adults)

Strengthens Bones & Muscles

Protects Against Pneumonia & Acute Respiratory Infections

Helps in Reducing Depression

Boosts Heart Health

Aids in Kidney Disease Treatment

RECOMMENDATION

General Recommendation on Preventive Screening

Rajesh
Kulkarni Booking ID :
5316220876

Risks Factors	Recommended Tests	Age Group (18-29 Yrs.)	Age Group (30-39 Yrs.)	Age Group (40-55 Yrs.)	Age Group (Above 55 Yrs.)
Diabetes	HbA1c Blood Glucose fasting	 Screen annually  Repeat earlier in case of symptoms  Under treatment- Repeat every 3-6 months	 Recommended  Screen annually  Repeat earlier in case of symptoms  Under treatment- Repeat every 3-6 months	 Strongly Recommended  Screen annually  Repeat earlier in case of symptoms  Under treatment- Repeat every 3-6 months	 Strongly Recommended  Screen annually  Repeat earlier in case of symptoms  Under treatment- Repeat Every 3 months
Thyroid Disorder	Thyroid Profile-Total (T3, T4 & TSH Ultra-sensitive)	 Screen annually  Repeat earlier in case of symptoms  Under treatment- Repeat every 3 months	 Recommended  Screen annually  Repeat earlier in case of symptoms  Under treatment- Repeat every 2-3 months	 Strongly Recommended  Screen annually  Repeat earlier in case of symptoms  Under treatment - Repeat every 2-3 months	 Strongly Recommended  Screen annually  Repeat earlier in case of symptoms  Under treatment- Repeat every 2-3 months
Vitamin-D Deficiency	Vitamin D Total 25-Hydroxy	 Recommended  Screen annually  Repeat earlier in case of symptoms  Under treatment - Repeat every 3 months	 Recommended  Screen annually  Repeat earlier in case of symptoms  Under treatment- Repeat every 3-6 months	 Strongly Recommended  Screen annually  Repeat earlier in case of symptoms  Under treatment- Repeat every 3-6 months	 Strongly Recommended  Screen annually  Repeat earlier in case of symptoms  Under treatment- Repeat Every 3 months
Vitamin B12 Deficiency	Vitamin B12 Cyanocobalamin	 Recommended  Screen annually  Repeat earlier in case of symptoms  Under treatment - Repeat every 3 months	 Recommended  Screen annually  Repeat earlier in case of symptoms  Under treatment- Repeat every 3-6 months	 Strongly Recommended  Screen annually  Repeat earlier in case of symptoms  Under treatment- Repeat every 3-6 months	 Strongly Recommended  Screen annually  Repeat earlier in case of symptoms  Under treatment- Repeat Every 3 months
High Cholesterol /Dyslipidemia	Lipid Profile Cholesterol-Total, Serum	 Screen annually  Repeat earlier in case of symptoms  Under treatment- Repeat every 3 months	 Recommended  Screen annually  Repeat earlier in case of symptoms  Under treatment- Repeat every 3 months	 Strongly Recommended  Screen annually  Repeat earlier in case of symptoms  Under treatment- Repeat every 3 months	 Strongly Recommended  Screen annually  Repeat earlier in case of symptoms  Under treatment- Repeat every 3 months
Kidney Disorder	Kidney function test Urine Routine & Microscopy Urea Serum	 Screen annually  Repeat earlier in case of symptoms  Under treatment- Repeat every 3 months	 Recommended  Screen annually  Repeat earlier in case of symptoms  Under treatment- Repeat every 3 months	 Strongly Recommended  Screen annually  Repeat earlier in case of symptoms  Under treatment- Repeat every 3 months	 Strongly Recommended  Screen annually  Repeat earlier in case of symptoms  Under treatment- Repeat every 3 months
Liver Disorder	Liver function test SGOT/AST SGPT/ALT	 Screen annually  Repeat earlier in case of symptoms  Under treatment- Repeat every 3 months	 Recommended  Screen annually  Repeat earlier in case of symptoms  Under treatment- Repeat every 3 months	 Strongly Recommended  Screen annually  Repeat earlier in case of symptoms  Under treatment- Repeat every 3 months	 Strongly Recommended  Screen annually  Repeat earlier in case of symptoms  Under treatment- Repeat every 3 months

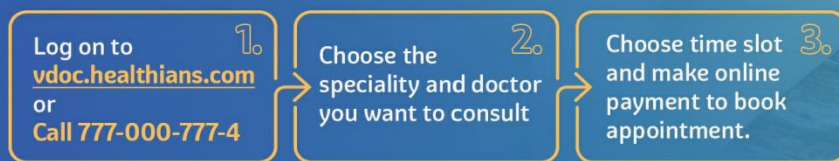


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About Healthians Labs

How we control Report Accuracy at Healthians



Quality Control

We follow Quality control to ensure both **precision & accuracy** of patient results.



Machine Data

We save patient's result values **directly from machines** ensuring no manipulations & no fake values.



QR Code

QR Code based authenticity check on all its reports



Calibration

We make use of calibrators to evaluate the **precision & accuracy** of measurement equipment.



Equipment

Our Labs are equipped with state-of-the-art instruments with **cutting edge technology** to provide faster & reliable results.



EQA

Our Labs participate in EQA & show proven accuracy by checking **laboratory performance** through external agency or facility.

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